

Request for a change in the way I work

Clause 10.3.2, Certified Agreement (No. 12) 2025 · s 27, Industrial Relations Act 2016 (Qld)

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

Employee reference MSH-INJ-5795

To: Injury Management, Human Resources, Logan Bayside Health Network

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Cc: LBH.HRTeam1@health.qld.gov.au

Mr Scott Hughes, Director, Corporate Services

Ms Chloe Taylor, Manager, Switchboard Services

Date: [SEND DATE]

1. WHAT THIS DOCUMENT IS

This is a request under clause 10.3.2 of the certified agreement for a change in the way I work.

I make it as a separate document, rather than within my response to the Request for Medical Information of 31 July 2026, so that there is no question about what it is or when it was received.

Clause 10.3.2 provides that, in accordance with the Industrial Relations Act 2016, an employee "may ask the employer for a change in the way the employee works, including the employee's ordinary hours of work". Section 27(1)(c) of that Act provides that the request may be for "a change to the way the employee works, for example, the use of different equipment as a result of a disability, illness or injury".

I am asking for a change in the way I work as a result of an illness. Clause 10.3.3 requires that the request be in writing, state the change in sufficient detail to allow a decision to be made, and state the reasons. Parts 2 and 3 below do each of those things.

2. THE CHANGE I AM ASKING FOR

2.1 MY ORDINARY HOURS

That my rostered pattern continue at the level at which it has in fact operated for the twelve months preceding the Employee Capabilities Checklist of 3 July 2026 — in the order of six shifts per fortnight.

I am not asking for a reduction. I am asking that the existing pattern continue. It was set by the Health Service, worked by me, and sustained across twelve months without deterioration. No concern was raised with me at any time that it was insufficient, and no request was made that I work more. The Employee Capabilities Checklist records it as "the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration", and as a "continuation of existing arrangement".

The Health Service holds the roster and payroll records from which the exact figure can be confirmed. I ask that it be confirmed from those records and, once confirmed, that the figure rather than my estimate be used.

2.2 THE WAY I WORK — ATTENDANCE CONFIRMATION

- (a) A telephone call one hour before each rostered shift, to confirm that I am attending.
- (b) If that call is not answered, or attendance is not confirmed, a second call.
- (c) If attendance is still not confirmed after the second call, the on-call operator is contacted and cover is arranged for the shift.

The measure is contingent, not standing. The on-call is engaged only if the two calls do not produce a confirmation, so it carries no cost on any shift I attend. On the shifts I do attend, which is most of them, the arrangement costs one telephone call.

An on-call arrangement already exists in the unit. On 15 April 2024 the team was advised that after-hours on call had been added, with the days highlighted on the roster and a contact process for urgent matters. What I am asking for is the application of a mechanism Switchboard Services already operates, not the creation of a new one.

To be clear about what this is and is not: it is a confirmation of attendance, of the kind any roster requires, made early enough to be useful. It is not an assessment of my condition, and I am not asking that colleagues on shift observe,

assess or report on my mental state. That would place a burden on staff who are not qualified to carry it, would require my health information to be known to co-workers, and would itself be a workplace stressor. No colleague needs to know anything about my health for (a) to (c) to work. They need only know whether the shift is confirmed.

2.3 THE WAY I WORK — IN-SHIFT ESCALATION

A defined escalation pathway which I may initiate during a shift if my condition is acutely exacerbated — including who assumes the board and how the shift is closed out — so that the response is planned rather than improvised.

2.4 THE WAY I WORK — COMPLAINT HANDLING

Continuation of the existing arrangement recorded in the Health Service's own correspondence: immediate escalation of complaints received by Switchboard staff to the Client Liaison Officer and/or the Manager, Switchboard Services. This is the Health Service's established process and it requires no change to anything.

2.5 THE WAY I WORK — CORRESPONDENCE ROUTING

That injury management correspondence concerning me be directed through the Injury Management team, Human Resources. This is what the Employee Capabilities Checklist itself recommends at Additional Comments 4(c): "return-to-work coordination and correspondence through the Injury Management team, Human Resources."

2.6 REVIEW

That the arrangement be reviewed at six weeks, or earlier if either party considers it necessary.

I am willing to discuss any of the above, and any alternative the Health Service or my treating practitioners consider more appropriate. If the Health Service would prefer a graduated commencement, I am willing to return on a graduated basis by agreement.

3. THE REASONS FOR THE CHANGE

3.1 The pattern has been sustained out of my own leave, and that is what has run out.

The six-shift roster was the arrangement the Health Service designed. Where my condition was exacerbated and I could not attend a rostered shift, I managed that myself, by taking sick leave or annual leave. No further adjustment was sought from the Health Service and none was provided. The variability was absorbed by me, out of my own entitlements.

That is why, on 3 July 2026, I wrote that my paid sick leave was exhausted. It was exhausted in the course of managing, without assistance, a condition arising in the workplace.

3.2 The change replaces that with a process.

The measures at 2.2 and 2.3 are directed precisely at that. They do not change the roster. They replace an arrangement under which exacerbations are absorbed by my personal leave balances with one under which they are managed by a defined process. That is what a control is, and it is what has been absent.

3.3 Distress peaks as a shift approaches, and the measures are directed at that.

Distress rises as a rostered shift approaches, and there have been occasions where I have called in at short notice as a result. What makes that worse — for the service and for me — is that a late call leaves the board short and the preceding shift unable to leave. The purpose of the sequence at 2.2 is that where I am not going to attend, that is established about an hour ahead rather than at or after start time.

3.4 It is directed at the only matter ever raised with me.

The only matter that has been raised with me in respect of my conduct is attendance and lateness. No concern about the quality of my work has ever been put to me, and the correspondence of 31 July 2026 states that the Health Service is not aware of any performance or conduct matters having been raised for appropriate management. The measures at 2.2 are directed at the one matter that has been raised, and are the measures most likely to prevent its recurrence. Had they been in place, they would have addressed a substantial part of the attendance history recorded in the correspondence of 8 September 2025. I raise that not to reopen it, but because it identifies the control that was missing.

3.5 The applicable policy contemplates exactly this.

HR Policy G03 provides at clause 1.1 that temporary adjustments exist so that an employee may "continue work during their recovery", and at clause 1 that managers are responsible for ensuring that work environments and work "do not unreasonably prevent employment for people". The adjustment contemplated by the policy is the thing that allows work

to continue while recovery occurs.

4. THE DECISION, AND WHO SHOULD MAKE IT

Clause 10.3.4 provides that the Health Service may grant this request, grant it in part or subject to conditions, or refuse it. Clause 10.3.5 provides that it may grant it in part or subject to conditions, or refuse it, "only on reasonable grounds".

I ask that the decision be made by an officer outside Corporate Services.

I do not ask that Corporate Services be removed from the management of my employment, and I make no allegation against any individual. Part of what this request concerns is the reporting arrangements within Corporate Services, and the officers who would otherwise decide it sit within those arrangements. Clause 1.2 of the Code of Conduct for the Queensland Public Service provides that a conflict of interest is not wrongdoing in itself, but that failing to disclose and manage one is likely to be wrongdoing. Section 89 of the Public Sector Act 2022 is to the same effect.

The separation I propose is the ordinary one: the clinical material held by Injury Management on a medical-in-confidence basis; the decision made outside Corporate Services; implementation — rostering, and putting in place whatever is decided — retained within Corporate Services. The officer steps out of the decision, not out of the workplace. This requires no restructuring and no finding to be made about anyone.

I ask that the officer who makes the decision be identified to me, together with the source of the delegation relied on.

5. THE TIME FOR THE DECISION

Clause 10.3.6 provides:

"The employer must give the employee written notice about its decision within 21 days after receiving the request. If the employer decides to grant the request in part or subject to conditions or to refuse the request, the written notice about the decision must state the reasons for the decision, outlining the reasonable grounds for granting the request in part or subject to conditions or for the refusal."

Section 28(3) of the Industrial Relations Act 2016 is in the same terms. Section 28(4) provides that a notice granting the request in part or subject to conditions, or refusing it, must state the written reasons outlining the reasonable grounds, and must state that the Commission has jurisdiction to hear and decide a dispute over the request under chapter 6. Section 29 provides that if the written notice is not given within 21 days after the request is received, the employer is taken to have decided to refuse the request.

Twenty-one days from the date of this letter is [21DAYDATE].

I record these provisions because a date has not been available in this matter at any stage. I have asked several times to be told when I would know whether I am returning to work, and I have not been given a date. The certified agreement provides one, and I would much prefer that the request be dealt with by agreement well before it.

6. WHAT THIS REQUEST DOES NOT DO

For completeness:

- (a) This request is separate from my response of the same date to the Request for Medical Information. The matters raised there are not made under clause 10.3 and are not answered by a decision under it.
- (b) This request is separate from any dispute notified under clause 1.11 of the certified agreement.
- (c) Nothing in this request is a concession that I am unfit to attend work. I remain willing and available to work, and have been since 3 July 2026. Identifying hazards and determining control measures is a function of the Health Service under Part 3.1 of the Work Health and Safety Regulation 2011, and nothing here relieves the Health Service of that function or is offered as a substitute for the workplace assessment I have asked for.
- (d) I make this request without waiver of any entitlement, and it is not to be treated as a request for leave of any kind.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

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